جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : نارمين انور راشد تاوضروس
المحافظة : القاهره **الحي / المدينة :** عين شمس **العنوان :** ~~
رقم البطاقة : 29112022601409 **رقم التليفون :** 01277419142 **تاريخ تسجيل الإستمارة :** 18-10-2026
العمر(تاريخ الميلاد) : 02-12-1991 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مسح PET/CT للجسم [8530] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: نارمين انور راشد تاوضروس
National ID: 29112022601409

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Whole-body PET/CT scan.**

التاريخ : **05-07-2026**	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

08/05/2026

MDT FORM

Name:	Narmen Anwar Rashed //	MRN:	2120
Age:	35 Y	ID:	29112022601409
Menopausal Sta	PRE	Phone:*	01277419142
NRS SCORE:		Marital Status:	
BMI:		Family History:	
Gender			
CO Morbidity:		Reason Of MDT:	
		metastatic BC	
Surgical History:*	MRM	ECOG Performance Status	
		◉ 0 ○ 1 ○ 2 ○ 3 ○ 4	

Clinical Data:

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: نارمين انور راشد **MR. :** 2120	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 34-year-old individual who is pre-menopausal. The clinical history is notable for a locally recurrent right operative bed breast cancer with metastasis to the liver. The patient underwent right modified radical mastectomy in 2018 for a triple-positive tumor and received adjuvant chemotherapy, radiotherapy, tamoxifen, zoladex and trastuzumab. In July 2023 a right local recurrence and liver metastasis were identified; initial PET-CT showed a single segment 4 hepatic lesion. The patient was treated with taxotere, trastuzumab, zoladex and anastrozole from February 2024, with trastuzumab continued until December 2025 when approval was withdrawn by the Ministry of Health. Subsequent CT in January 2026 demonstrated resolution of the hepatic lesion, but a new right breast mass was identified on MRI involving the operative bed, extending to the nipple, pectoralis muscle and right axillary lymph nodes, raising suspicion for local progression. A repeat CT in December 2024 had shown complete response of the segment 4 hepatic lesion.

A tru-cut biopsy of the right breast mass confirmed invasive ductal carcinoma, grade 3, with immunohistochemistry revealing ER 6/8, PR 0/8, HER2 3+, and Ki-67 35%. PET-CT performed in March 2026 demonstrated progression of the right breast mass while maintaining complete response of the hepatic lesion. The patient's trastuzumab supply is no longer approved by the Ministry of Health; a trial to obtain trastuzumab from an NGO is ongoing. The patient received three cycles of taxotere, the last on 2 June, then was switched to capecitabine (xeloda) and lapatinib (tykerb) as an anti-HER2 protocol while awaiting NGO support; only one cycle of capecitabine was administered on 14 July. CT performed on 4 July showed no pulmonary or hepatic metastases, sustained complete response of the hepatic lesion, and no detectable breast mass on imaging or clinical examination, although a D9 lytic bone lesion was noted on bone scan.

The referring team recommended that, if the patient remains free of disease, a pseudo-neoadjuvant regimen of taxotere plus trastuzumab with or without pertuzumab be considered, followed by right-sided mastectomy. The MDT outcome and plan is to proceed with taxotere combined with Phesgo (trastuzumab-pertuzumab-pertuzumab-derivative) now approved by the NGO, with regular assessments of therapeutic response.

تحريرا في ,

التاريخ :	23-09-2026	**الوقت :**	

الطبيب المعالج :

يعتمد ,

BX:

ER: **PR:** **HER2:** **KI67:**

History:

locally recurrent (rt operative bed) met to liver
history rt MRM 2018 triple positive adj chemo port tam zoladex herceptin
.7/2023 rt local recurrence and mets to liver - initial pet ct 7/2023 single seg 4 hfl rec taxotere herceptin st zoladex arimidex from 2/2024 and herceptin last12/2025 (no more approval by moh) cts 1/2026 NO HFL, newly developed rt breast mass for MRI. MRI breast rt operative bed mass reaching nipple and pectoralis in rt axillary LN, suspected local PD for MDT. CTs from 12/2024 CR of seg 4 HFL

last MDT > In view of prolonged PFS....for trial of PET/CT scan...if free, consider pseudo-NACT Taxotere/Herceptin +/- Perjeta. then consider Rt SM.

PET CT 3/2026 > PD of rt breast mass and CR of HFL
tru-cut biopsy of rt breast mass IDC3 ER6/8 PR0/8 HER2 3+ Ki67 35%

herceptin no longer approved by MOH, with trial to receive herceptin from NGO, pt st tx with taxotere, received 3 cycles, last in 2/6, Then shifted to xeloda tykerb for an anther2 protocol while awaiting NGO.. now phesgo is provided from NGO

pt received only 1 cycle xeloda on 14/7

CTs 4/7 > No pulm or hepatic mets (maintained CR of HFL) and no breat mass detect on CT or clinical exam. D9 lytic lesion for bone scan

Sonomammography:

Biomarker/Molecular

Mets Workup:

Recommendations:

MDT Outcome & Plan

Pt has approval of Phesgo from NGO...go for Taxotere/Phesgo with regular assessment of response.



TNM

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY

MDT Form
[[DOCTYPE:MDT_FORM]]

Patient's Name:	**Narmin Anwar Rashid**
Date:	**2026-07-04**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

MULTISCLICE C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS AFTER I.V. CONTRAST INJECTION REVEALED:

Known case of breast CA underwent bilateral MRM with breast implants noted.

I- CHEST:

- The left breast implant shows wavy outline for sonographic assessment.
- Right upper lobe subpleural reticulations likely post radiation sequelae.
- No pulmonary nodules, masses, consolidation, collapse or cavitary lesions.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Mildly enlarged liver showing homogenous parenchymal attenuation. No focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness, nephrogram and excretory function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, adrenal glands, pancreas, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Left adnexal cyst is seen measuring 67 mm.
- IUCD is seen within the uterus.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- D9 vertebral body tiny lytic focus.

OPINION:

- **No pulmonary or hepatic metastatic lesions.**
- **D9 vertebral body tiny lytic focus for bone scan.**

Dr.sherif Wagdy

Dr.sherif Wagdy

CT Scan
[[DOCTYPE:CT_SCAN]]

Entered by: 467 On: 2026-06-15

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

06/15/2026

MDT FORM

Name:	Narmen Anwar Rashed //	**MRN:**	2120
Age:	34 Y	**ID:**	29112022601409
Menopaus Status:	premeno	**Phone:**	01277419142
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	+VE MOTHER,SISTER
CO Morbidity:	free	**Reason Of MDT:**	RE MDT

Clinical Data:

BX:

ER 6/8 **PR** 0/8 **HER** 3+ **KI6:** 35%

Sonomammography:

LOCAL RECCURENT BREAST CANCER AFTER SSM +ALND +IMPLANT 2018

Mets Workup:

R L



T N M

History:

locally recurrent (rt operative bed) met to liver .history rt mrm 2018 triple positive adj chemo port tam zoladex herceptin .7/2023 rt local recurrence and mets to liver initial pet ct 7/2023 single seg 4 hfl rec taxotere herceptin st zoladex arimidex from 2/2024 and herceptin last12/2025 (no more approval by moh) cts 1/2026 newly developped rt breast mass for mri no hfl .mri breast rt operative bed mass reaching nipple and pectoralis m rt axillary ln .suspected local pd for mdt .cts from 12/2024 cr of seg 4 hfl mets

last MDT > In view of prolonged PFS....for trial of PET/CT scan.../ if free, consider pseudo-NACT Taxotere/Herceptin +/- Perjeta. then consider Rt SM.

PET CT > PD of rt breast mass and CR of HFL
tru-cut biopsy of rt breast mass IDC3 ER6/8 PR0/8

Recommendations:

FOR TYKERB? meanwhile trial of herceptin at NGO

MDT Outcome & Plan

tykerb xeloda meanwhile surgical consultation if feasible surgery

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT Form
[[DOCTYPE:MDT_FORM]]

Cairo University
National Cancer Institute
Department of Diagnostic and
Interventional Radiology

جامعة القاهرة
المعهد القومي للأورام
قسم الأشعة التشخيصية والتداخلية

Exam Information:

Patient Name: Narmeen ANWAR RASHED	*Date of birth:* 02/12/1991
Arabic Name: نارمين انور راشد	*Patient Gender:* F
Patient RIS ID: 380818 *Patient HIS ID:* 10110226000854	*Exam Date:* 05/02/2026
Written by: Dr.Karam Gaber	**Approved by:** Dr.Karam Gaber

MRI BREAST EXAMINATION

TECHNIQUE:

-Axial T2, STIR and DWIs with limited dynamic study and subtraction.

FINDINGS:

- Known case of recurrent RT breast cancer with liver metastasis on CTH as mentioned by PT . no available previous studies for comparison
- Current study revealed mild oedma pattern with irregular spiculated heterogeneously enhancing central mass lesion reaching nipple anteriorly and pectorial muscle posteriorly , lesion collectively measuring about 5x7x 3.5 cm likely representing the known recurrence
- Ipsilateral multiple pathological discrete level 2 axillary LNS
- Bilateral silicon implant showing mild contracture on left side and right side on right side
- No gross suspicious mass or non mass enhancement on left side .
- Normal skin thickness and contour on left side .
- Normal nipple-areola complex on left side .
- No gross left sided pathological enhanced axillary lymph nodes.

OPINION:

-Normal MRI study of left breast.
Known case of recurrent right breast cancer with changes as described

M.R.I
[[DOCTYPE:MRI]]

Name: Narmin Anwar Rashed | Date: 3/3/2026

Referred by: Prof; | MRN: 30168748

F18-FDG PET/CT STUDY

CLINICAL HISTORY:

- *A known case of managed breast cancer post MRM.*

REASON FOR PET/CT STUDY:

- *Assessment of therapy response.*

TECHNIQUE:

- Radiopharmaceutical: 444 MBq of 18F FDG.
- Time from 18F-FDG injection to scan: 40 min
- PET/CT images were acquired from the skull base through the upper thighs & CT images were acquired for the purpose of anatomical localization and attenuation correction. Images were reviewed in the transaxial, coronal, and sagittal planes.
- Additional notes: SUV max normalized to body weight was used.
- Comparison: Previous PET/CT dated 6/8/2023.

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202) 25 35 1745
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١ شارع سكة الإمام - السيدة زينب - القاهرة
ص . ب : ١ الدواوين - مصر
الهاتف : 19057 (202)
فاكس : 25 35 1745 (202)
57357Egypt

AFNCI
جمعية أصدقاء المكافحة الدولية ضد السرطان
57357 Egypt Cancer Network USA
57357 Egypt Cancer Network Canada
57357 Egypt Cancer Network Switzerland

PET Scan
[[DOCTYPE:PET_SCAN]]

FINDINGS:

Head and Neck:

* No hypermetabolic cervical lymphadenopathy detected.
* NO FDG avid thyroid nodules.
* Physiologic FDG uptake is seen in the oropharynx, salivary glands, and larynx.
* Normal diffuse brain activity with NO hypermetabolic brain lesions.

Loco-regional:

Status post right MRM with bilateral breast implants

* Right breast with interval size and metabolic progression of the previously seen FDG avid UOQ soft tissue nodule measures 2x1.7 cm of SUVmax~10.6 compared to 1.8 x 1.3 cm of SUVmax~5.9 previously. With currently seen tiny adjacent FDG avid nodule laterally, measures about 7 mm of SUVmax~3.9
* Left breast implant shows irregular outlines for MRI assessment.
* No pathologically enlarged FDG avid right axillary LNs.
* Left axillary LNs with preserved fatty hila still showing no significant metabolic activity ? reactive.

Chest:

* There is no hypermetabolic mediastinal lymphadenopathy.
* No FDG avid pulmonary nodules
* Right upper and middle lobe subpleural reticulations. Post radiation
* No metabolically active pericardial or pleural lesions detected.
* Physiologic FDG uptake is noted in the heart.

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AFNCI
57357 Egypt Cancer Network USA
57357 Egypt Cancer Network Canada
57357 Egypt Cancer Network Switzerland

PET Scan
[[DOCTYPE:PET_SCAN]]

Abdomen and Pelvis:

* **The liver is of enlarged size with fatty changes. With total resolution of the previously seen left lobe FDG avid hypodense focal lesion at segment IVb with no sizable FDG avid residual**
* There is no hypermetabolic abdominal or pelvic lymphadenopathy.
* The spleen is of average size with NO FDG avid focal lesions detected.
* The pancreas shows normal FDG avidity.
* There are no active adrenal nodules.
* Physiologic FDG excretion is seen in the kidneys and bladder.
* No ascites.
* **Stationary left adnexal cyst measures 7x4.2 with no gross mural nodule for US assessment.**
* **RVF uterus with IUD see in situ.**

Musculoskeletal:

* Normal FDG activity is seen over the axial skeleton. No focal active blastic or lytic bone lesions detected.

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جمعية أصدقاء الحضارة الفرعونية ضد السرطان
Egypt Cancer Network USA
Egypt Cancer Network Canada
Egypt Cancer Network Switzerland
PET Scan
[[DOCTYPE:PET_SCAN]]

Patient Name	:الأستاذه / نارمين انور راشد ناوضرس	Request No.	: 13005405
Patient No.	:29112022601409	Request Date	: 10/03/2026
Daily No.	:676/26	Verification Date	: 17/03/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	:34 Y	Department	: Out Patient
Referring Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right breast mass.
Nature of Specimen: Tru-cut biopsy, collected on 10/3/2026, at 12:45, Cold ischemia time: 20 seconds.

Gross Examination:
Four grey white tissue cores, ranging in size between 1.0 cm & 1.5 cm long, totally submitted.

Microscopic Examination:
- Histological type: Invasive duct carcinoma.
- Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- Ductal carcinoma in situ: Absent.
- Perineural invasion: Absent.
- Angioinvasion: Present.

Diagnosis:
Right breast mass, Tru-cut biopsy:
- Invasive duct carcinoma, Grade 3.
- For further IHC (ER, PR, Her2, Ki67).

Signature
Dr. Walaa Ibrahem Ghonaim.

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / نارمين انور راشد تاوضرس	Request No.	: 13005405
Patient File.	: 29112022601409	Request Date	: 10/03/2026
Path No.	: 676/26	Verification Date	: 17/03/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 34 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass, diagnosed invasive duct carcinoma"Grade 3".

Nature of Specimen: Tru-cut biopsy, collected on 10/3/2026, at 12:45 , Cold ischemia time: 20 seconds.

Technique:

Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

ER: (All controls showed appropriate reactivity):
ER: Strong nuclear reaction in 15% of tumor cells (score 6/8).
PR: Negative nuclear reaction in tumor cells (score 0/8).
Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
Ki67: 15%.

Conclusion:

Right breast mass, Tru-cut biopsy:

ER: Positive (score 6/8).
PR: Negative (score 0/8).
Her2: Positive (score 3+).
Ki67: 15%.

Scoring system: "ER, PR"
According to ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, less than 1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2,

Verified by :
Dr.Duaa Mohamed

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Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	الأستاذه / نارمين انور راشد تاوضرس	Request No.	13005405
Patient No.	29112022601409	Request Date	10/03/2026
File No.	676/26	Verification Date	17/03/2026
Sex	Female	Financial Category	جوستاف روسي - نفقة الدولة
Age	34 Y	Department	Out Patient
Referring Doc.		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Intermediate: and 3, strong). Total score from 0 to 8.
Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within > 10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense in > 10% of tumor cells.

Signature:
Dr. Walaa Ibrahem Ghonaim.

Verified by :
Dr.Duaa Mohamed

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Immunohistochemistry
[[DOCTYPE:IHC]]

[[DOCTYPE:MDT_FORM]]
MDT Form

جوستافروسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

10/14/2026

MDT FORM

Name:	Narmen Anwar Rashed //	**MRN:**	2120
Age:	35 Y	**ID:**	29112022601409
Menopausal Sta	pre	**Phone:**	01277419142
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
CO Morbidity:		**Reason Of MDT:**	

Clinical Data:

BX:

ER: **PR:** **HER2:** **KI67:**

Sonomammography:

Mets Workup:

History:

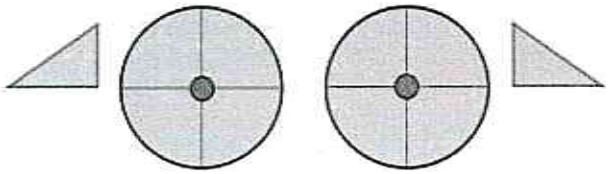
locally recurrent (rt operative bed) met to liver
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herceptin no longer approved by MOH, with trial to receive herceptin from NGO, pt st tx with taxotere, received 3 cycles, last in 2/6, Then shifted to xeloda tykerb for an antiher2 protocol while awaiting NGO.. now phesgo is provided from NGO
pt received only 1 cycle xeloda on 14/7
CTs 4/7 > No pulm or hepatic mets (maintained CR of HFL) and no breat mass detect on CT or clinical exam. D9 lytic lesion for bone scan
bone scan 8/2026 >> free
CT CAP 30/9>> SD except newly developed precaval lns 20*14mm for

Recommendations:

MDT Outcome & Plan

In View of the Liver being the only metastatic site (Now CR).
For Counseling the Patient regarding RT SM (preferably after 1-2 more cycles of PHESGO) then pseudo adj ttt acc to operative pathology

R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د نشوي حسن	د اسيه عيسوي	د نرمين رمضان			

14/10/2026